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BSH GOOD PRACTICE PAPER

Implementation and value of providing a multidisciplinary haematology and gynaecology service for individuals with bleeding and thrombotic disorders: A British Society for Haematology & United Kingdom Haemophilia Centre Doctors' Organisation Good Practice Paper

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abnormal uterine bleeding, anti-coagulation, gynaecological management, heavy menstrual bleeding, inherited bleeding disorder, multidisciplinary care, ovulation-related bleeding, venous thromboembolic disease

METHODOLOGY

This Good Practice Paper was compiled according to the British Society for Haematology (BSH) process at https://b-s-h.org.uk/media/or2db2zv/2024-08-20-bsh-guidelines-development-process.pdf?cf=638372962768270000. The BSH produces Good Practice Papers to recommend good practice in areas where there is a limited evidence base but for which a degree of consensus or uniformity is likely to be beneficial to patient care. The Grading of Recommendations Assessment, Development and Evaluation (GRADE) nomenclature (http://www.gradeworkinggroup.org) was used to assess the strength of the recommendations.

LITERATURE REVIEW DETAILS

A literature search was performed on 27 November 2024 (and updated on 6 March 2026) and included searches of EMBASE, Ovid MEDLINE, Cumulative Index to Nursing and Allied

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Health Literature (CINAHL) Ultimate and The Cochrane Library databases. Details of the search can be found in Appendix S1. Due to the paucity of published data, all types of publication, including editorials, case reports and conference abstracts, were included. Those relating to non-human studies and published in non-English language journals were excluded.

REVIEW OF THE MANUSCRIPT

The writing group produced the draft guideline. Review of the manuscript was performed by the BSH Haemostasis and Thrombosis task force, the BSH Guidelines Committee, BSH sounding board and the United Kingdom Haemophila Centre Doctors' Organisation (UKHCDO). It has also been reviewed by the UK Haemophilia Society, Haemophilia Nurses Association, Royal College of Obstetricians and Gynaecologists (RCOG) Guidelines Committee, RCOG Women's Network, Thrombosis UK, Haemophilia Wales, Haemophilia Scotland and the Royal College of General Practicioners. These organisations do not necessarily approve or endorse the contents.

INTRODUCTION

The management of gynaecological issues forms a significant part of the care of girls and women with haematological diagnoses, with differing needs relating to specific life stages such as menarche, need for contraception, fertility, pregnancy and management of menopause. There is an overlap in presentation to haematology and gynaecology services of individuals with heavy menstrual bleeding (HMB). Bleeding disorders, inherited or acquired, or the use of anti-coagulant or anti-platelet therapy can be complicated by HMB or other types of abnormal uterine bleeding (AUB). Corpus luteal haemorrhage (CLH) may also occur.[1] These may be the first presentation of a bleeding disorder or an exacerbation of pre-existing problems. A previous history of thrombosis, or the presence of thrombotic risk factors, may influence choices in relation to the hormonal management of contraception, HMB or menopausal symptoms.[2] Provision of a multidisciplinary haematology and gynaecology service for individuals with bleeding and thrombotic disorders, whereby patients are seen jointly by specialists from both specialties, or there is communication between specialties, with management by clinicians interested in this area, has the potential to offer best clinical care and cross-specialty education.[3] This good practice paper is relevant to clinicians, clinical nurse specialists and other multidisciplinary team (MDT) members in haematology, particularly haemostasis and thrombosis and gynaecology. It is also relevant to those who are responsible for developing patient-centred services, such as service managers and healthcare commissioners.

DEFINING THE POPULATION

The following groups are considered within this good practice paper as being appropriate for evaluation by a multidisciplinary haematology and gynaecology service:

(i) Individuals with an inherited or acquired bleeding disorder, including immune thrombocytopenia (ITP), who are experiencing HMB;
(ii) Pre-pubescent females with an inherited or acquired bleeding disorder in whom HMB is anticipated at menarche;
(iii) Adolescent females who present with HMB from menarche, which may be the first presentation of a bleeding disorder;
(iv) Individuals receiving anti-coagulant or anti-platelet therapy (with thrombotic risk factors or history of thrombosis) experiencing HMB;
(v) Individuals with complex thrombotic risk requiring discussion of contraceptive choice or management of HMB or menopausal symptoms.

Although most individuals who are referred from primary care with HMB are seen in a gynaecology clinic first, to facilitate appropriate gynaecological examination/imaging, adolescents with HMB are a priority for early multidisciplinary management due to the high prevalence of an underlying bleeding disorder diagnosis. These decisions are likely to be influenced by the presenting history, including the presence or absence of additional haemorrhagic symptoms. Centres should also consider how to ensure that those presenting with severe HMB via the emergency department (ED) have equitable access to the benefits of a combined service to those who are referred as outpatients.

Individuals with malignancies who require aggressive cytotoxic treatment are likely to develop significant HMB due to thrombocytopenia. Treatment to suppress menstruation is routine practice in most centres and therefore this group are not included in this guidance as recommendations for their management are published elsewhere.[4,5] Centres may choose to include within their service the management of individuals with an acquired bleeding tendency as a result of a systemic disorder, for example, liver or renal disease, but we have narrowed the scope of this Good Practice Paper to those with conditions which are usually managed within an outpatient haematology setting, to match clinical practice within the United Kingdom.

Although we use the terms 'women' and 'girls' within this document, we recognise that it is not only individuals who identify as women who may be menstruating.

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SUMMARY OF THE EVIDENCE TO SUPPORT THE DEVELOPMENT OF A MULTIDISCIPLINARY HAEMATOLOGY AND GYNAECOLOGY SERVICE

HMB affects up to 25% of all women and girls and is a common presentation to gynaecology clinics.[3] At least 10% of these individuals are subsequently diagnosed with an inherited bleeding disorder (IBD),[1] this figure being highest in adolescents presenting with HMB (up to 33% in some studies).[6] The most frequently diagnosed IBDs in this setting are von Willebrand disease (VWD) and platelet function disorders, both of which can have normal basic coagulation results. A high index of clinical suspicion is therefore crucial to ensure that an underlying IBD is not missed. A barrier to accurate diagnosis is a lack of awareness in gynaecological care providers regarding bleeding disorders and how to investigate for them with evidence suggesting that only 15% of gynaecologists would consider an IBD during initial consultation for HMB.[7,8]

Approximately 55% of girls and women with bleeding disorders (GWBD) report HMB.[9] Up to 70% of menstruating females receiving anti-coagulant therapy experience HMB,[10] many of whom require longer term or lifelong anti-coagulation due to ongoing thrombotic risk factors. Suppression of HMB in females with abnormal haemostasis can be challenging, requiring a median of three strategies for optimal response.[11,12] Invasive gynaecological procedures in patients with IBD or on anti-coagulant therapy are also more complicated than in individuals with normal haemostasis.

Collaboration between haematologists and gynaecologists is required to determine optimal therapeutic strategies, balancing the relative risks of hormonal, haemostatic and anti-coagulant treatments and to ensure that invasive procedures are performed safely.

Providing carefully considered treatment options within the context of a multidisciplinary haematology and gynaecology clinic facilitates the development of personalised care plans that align with patient preferences, minimise side effects and optimise outcomes.[13] Patients report a reduction in symptoms such as HMB,[14] and being seen in the clinic can lead to a prompt diagnosis of bleeding disorders.[15] GWBD value these clinics, reporting high levels of satisfaction,[16–18] and appreciation of the multidisciplinary nature, with over 70% of patients preferring this structure of care.[18] Services offering them find them to be both practical and convenient.[19]

Among European haemophilia centres without a combined clinic, 66% surveyed considered it would be beneficial to establish one. In services where there are barriers to developing these clinics, centres have identified these as stemming from institutional, financial or physician-imposed factors, despite patient support for them.[20] A multidisciplinary approach with good communication between specialties is highlighted as an implementation action in the European Association for Haemophilia and Allied Disorders (EAHAD) principles of care.[21] In the United Kingdom, the 2025 Haemophilia Society report 'Underserved-Overlooked: How our health systems are failing women and girls with a bleeding disorder' [https://haemophilia.org.uk/underserved-overlooked/] has recommended that regional bleeding disorder networks include gynaecologists in the MDTs and that haemophilia centres run joint bleeding disorders and gynaecology clinics for patients that need them. In addition, the peer review assessment for UK haemophilia centres includes evidence of multidisciplinary discussion with obstetrics and gynaecology as a quality standard [https://www.ukhcdo.org/wp-content/uploads/2024/09/Peer-Review-Quality-Standards-2023-v4.0.pdf]. Despite these recommendations, a recent UKHCDO survey of UK bleeding disorders centres has identified inequity of access to a multidisciplinary service, with 78% not having a joint bleeding disorders and gynaecology clinic.[22] In the United States, the Foundation for Women and Girls with Blood Disorders has identified a number of centres of excellence for their interdisciplinary management of menstruating females receiving anti-coagulant therapy and has published a recommended toolkit for the development of a collaborative clinic.[23]

GUIDANCE FOR THE DEVELOPMENT OF A MULTIDISCIPLINARY HAEMATOLOGY AND GYNAECOLOGY SERVICE

The optimal service model will be dependent on factors specific to the region served, such as the local geography, population demographics, available facilities and the expertise of available staff. Table 1 highlights the key points that should be considered in setting up and operating a multidisciplinary service.

A GUIDE TO THE MANAGEMENT OF INDIVIDUALS ATTENDING A MULTIDISCIPLINARY HAEMATOLOGY AND GYNAECOLOGY SERVICE

Women with IBDs or receiving anti-coagulation are prone to AUB that comprises heavy and/or unscheduled bleeding. In addition to vaginal bleeding, this group of women can present with bleeding and pain induced by ovarian activity, for example. pelvic haematomas subsequent to ovulation or haemorrhagic functional cysts/CLH. HMB may be the main presenting symptom but management should also take into account related cyclical symptoms such as pelvic pain and pre-menstrual symptoms.[24]

Medical treatments for common gynaecological conditions are often hormonal and the thrombogenic potential of such agents in women at risk of venous thromboembolism

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TABLE 1 Key points to consider in the development of a multidisciplinary haematology and gynaecology service.

Clinical Review Model	Commonly used formats for patient reviews and development of interdisciplinary management plans include: • Joint in-person clinic consultations • Joint remote (video/telephone) clinic consultations • Hybrid in-person or remote clinic consultations, for example, remote follow-up consultations • MDT meetings for case-based discussion
Healthcare Team	Essential team members: • Gynaecologist • Haematologist, with an interest in haemostasis, thrombosis and anti-coagulation • Clinical nurse specialist in haematology and/or gynaecology Desirable team members: • Adolescent gynaecologist • Paediatric haematologist The role of essential team members, including training and administrative duties, should be part of their formal job description.
Clinic Environment	*Location*: Proximity to an accredited coagulation laboratory, e.g. at a hospital that has a bleeding disorders service. *Clinic area*: Should provide adequate privacy and a separate area for adolescent patients is preferred. *Clinic room(s)*: Spacious, with facilities for gynaecological examination, portable pelvic ultrasound device.
Clinical Nurse Specialist Role	The clinical nurse specialist acts as a primary point of contact for the patients before, during and after clinic appointments, ensuring continuity of care and building a relationship of trust. They should assist with: 1. Explanation of menstrual diaries, mobile apps, and quality-of-life tools 2. Supporting patients during clinical examination and investigations 3. Informing patients about treatment options and impact on fertility 4. Facilitating peer support groups and signposting to other agencies for information and support 5. Sharing information with schools and workplaces as appropriate, e.g. toilet pass for adolescent at school
Supporting Services	Essential supporting services: • Phlebotomy • Haematology and coagulation laboratory for blood investigations • Microbiology/virology laboratory for screening for sexually transmitted infections • Haematology/medical day unit for iron infusion Desirable supporting services: • Youth worker • Reproductive medicine/fertility service • Psychosexual support service • Pelvic floor therapist • Endometriosis clinic
Patient Pathway	Preparation prior to attendance: Patients should receive guidance on what to expect, including the possibility of requiring an intimate examination. Adolescents should be invited to bring a family member they are comfortable with. A (female) interpreter should be arranged if the patient has been identified as requiring one. Assessments prior to attendance: *Menstrual diary*: A mobile app, paper diary or quality of life (QoL) questionnaire, e.g. Menorrhagia Multi-Attribute Scale (MMAS),[24–26] can be used to document menstrual products used, frequency of changing them, clots and flooding, duration of menstrual bleeds and impact on QoL. *Blood tests*: Full blood count, ferritin level and basic coagulation screen may be appropriate prior to attendance. Further testing, e.g. von Willebrand screen and factor assays, will depend on test availability at the referring site, and it may be sensible to defer this until multidisciplinary clinic review in order to avoid pre-analytical errors due to problems with sample storage, transport and delay in processing.[27] *Gynaecological investigations*: Pelvic ultrasound scan, cervical smear test, STI screen may be appropriate prior to attendance. Multidisciplinary clinic assessment: *Gynaecological history*: Focus should be on QoL rather than bleeding severity.[26,28] *Bleeding history*: A thorough bleeding history to identify other haemorrhagic symptoms and family history, which may indicate an underlying IBD. *Thrombotic history*: Including previous thrombotic events, previous/ongoing thrombotic risk factors and family history. *Medication history*: Including anti-platelet, anti-coagulant, haemostatic and hormonal treatments. Follow-up assessments: *Response to interventions*: Including change in menstrual symptoms, effect on QoL and reassessment of iron status.
Online Assessment Tools	Several free apps* for tracking periods and resources for women with bleeding disorders are available and include: Talking Red: https://haemophilia.org.uk Know Your Flow: https://knowyourflow.ie These tools can be useful for providing information for patients and monitoring response to treatments. *These were available at the time of writing and are apps that have been developed with input from patients.

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TABLE 1 (Continued)

Liaison with other care providers	The following are important considerations for communication with other care providers: 1. Eligibility criteria for referral to the service should be clear, documented and shared with potential referrers* 2. Results of investigations should be shared with the General Practitioner and others involved in the care of the patient 3. Suggested therapies and changes in medications should be communicated clearly. These should follow local formulary guidance where possible. For off-licence use or deviation from standard dosing or frequency, medications may need to be prescribed in secondary care 4. Education should be provided for referrers to optimise initial assessment, diagnosis and management, along with appropriate and timely referral to the multidisciplinary service *The use of a standardised clinic referral proforma might be helpful. Ideally, this would be electronic and partially auto-populated.

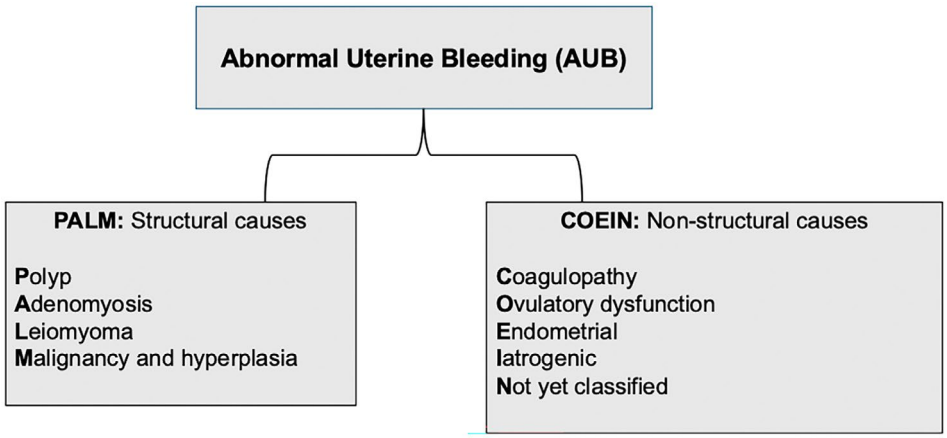


FIGURE 1 The PALM-COEIN classification that defines structural and non-structural causes of abnormal uterine bleeding.[31]

(VTE) may need to be considered.[2] The use of oestrogen-containing preparations can be considered in women who are anti-coagulated.[29] Progesterone-only forms are acceptable regardless of anti-coagulation status but caution is needed in women with additional thrombotic risk factors if high dose oral progestogens are being considered for use in the medium to longer term.[30]

If surgical or radiological options for management are indicated in individuals with abnormal haemostasis, careful planning is necessary to optimise the patients' iron status prior to surgery, prevent excessive peri- or post-operative bleeding and minimise any thrombotic risk from interrupting anti-coagulation.

Management options should take account of whether the individual has plans for pregnancy, and the timing of such and whether there is a desire to retain fertility.

Heavy menstrual bleeding

Management of HMB should be evidence-based and in accordance with published guidelines where available, such as the National Institute for Health and Care Excellence (NICE) guideline for the management of HMB.[24] Although women with a bleeding propensity related to an IBD or anti-coagulation are more likely to have HMB, it remains important to identify any potential gynaecological causes for their HMB. These can be broadly categorised according to the PALM COEIN (Polyp, Adenomyosis, Leiomyoma [fibroids], Malignancy and hyperplasia, Coagulopathy, Ovulatory dysfunction, Endometrial, Iatrogenic, Not yet classified) classification that defines structural and non-structural causes (Figure 1).[31] Appropriate gynaecological management options should be broadly the same for women with abnormal haemostasis and those without. However, the former group is more likely to be refractory to conventional gynaecological treatments and to have already tried first-line haemostatic management such as tranexamic acid, and so increased follow-up to gauge outcomes and changes or additions to gynaecological treatments may be necessary.[32]

Individuals with inherited bleeding disorders

Reference should be made to previously published guidance on the gynaecological management of GWBD.[1] Extended use of combined oral contraceptive pill (COCP), or the use of progesterone-only preparations at increased dose/frequency or reduced interval, are frequent approaches. A levonorgestrel releasing intrauterine system (LNS-IUS, e.g. Mirena™) can be inserted in non-sexually active/younger females with the aid of clinical expertise, in an appropriate clinical setting and sometimes with sedation/anaesthesia. There are reports of increased expulsion rates in women with IBDs, and the role of menstrual suppression to decrease this risk has been described.[33] The cyclical use of haemostatic products, for example, desmopressin, clotting factor concentrate or platelet transfusion, may be

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required to control problematic bleeding unresponsive to hormonal approaches. Further details on the management of HMB in individuals with bleeding disorders are available in Table S1.

Individuals receiving anti-coagulant therapy

Anti-coagulant agents may need to be changed in treatment resistant cases, for example, to apixaban or dabigatran which have a lower incidence of HMB.[34] Continuation of COCP during anti-coagulation may be appropriate in some cases with a clear plan to stop, with an appropriate crossover period, if a course of anti-coagulation is completed or interrupted. Interruption/reduction of anti-coagulant therapy during menstrual bleeds is an approach that should be used with caution as it may be associated with risk of recurrent VTE, but may be suitable in some cases.[35] Further details of approaches to managing HMB in anti-coagulated individuals are available in Table S1.

Managing thrombotic risk

Key considerations in managing individuals with increased thrombotic risk, due to personal history of thrombosis and/or presence of additional thrombotic risk factors, are available in Table S1. The risks versus benefits of oestrogen-containing oral contraceptives and hormone replacement therapy (HRT), including differing routes of administration (e.g. transdermal, transvaginal) where available, require careful interdisciplinary consideration.[2]

Acute gynaecological presentations

Women may present as emergencies with HMB, genital tract trauma (e.g. vaginal tears, vulval haematomas), miscarriage and intra-abdominal bleeding from early pregnancy-related complications (e.g. ectopic pregnancy) and non-pregnancy related pathologies, for example, haemorrhagic ovarian cysts, ovulation or sexual trauma. Diagnostic work up and treatment should follow standard gynaecological practice.[24] However, it is essential to recognise an individual with an IBD or receiving anti-coagulant therapy promptly in an emergency presentation and contact a haematologist urgently for advice on haemostatic support.[36]

Surgical intervention

Haemoglobin concentration and iron status should be optimised prior to the procedure where possible, although in situations where uterine pathology is contributing to iron deficiency it may be necessary to accept the risks of undertaking surgery when anaemic in order to offer a definitive solution. In cases of persistent severe anaemia, pre- or peri-operative red cell transfusion can be considered but should be in line with institutional and national transfusion guidelines.

Existing guidance on the gynaecological management of GWBD recommends close liaison between the gynaecological surgeon, anaesthetist and haematologist.[1] Specific haemostatic support with anti-fibrinolytics, desmopressin, and/or clotting factor concentrates and blood products will depend upon the particular IBD, its severity, and the likelihood of peri- and post-operative bleeding according to the nature of the gynaecological procedure.

For anti-coagulated women it is important to formulate a clear peri-operative anti-coagulation plan, including when to reintroduce anti-coagulation. Haematological liaison with anaesthetists regarding the appropriateness of neuraxial anaesthesia in anti-coagulated women and those with IBDs may be necessary. The blood transfusion laboratory and the haematology team should be alerted for high bleeding risk elective and emergency surgeries.

Women without a bleeding or thrombotic disorder who have significant fibroids can present a challenge to gynaecologists to adequately suppress menstruation and achieve a satisfactory pre-operative haemoglobin concentration. Such women often have repeated postponement of surgical intervention. Adding in a bleeding tendency can mean that an even greater challenge is presented. Multidisciplinary input and timely medical management may be necessary to allow curative surgery to be performed, taking into account the risks of repeated deferral. Further details on management of this clinical scenario are available in Table S1.

The management of termination of pregnancy for women with bleeding or thrombotic disorders can be delayed by referral to providers operating outside of a hospital setting. These individuals may not be suitable for care in this environment and should be referred to the appropriate hospital site for multidisciplinary care.

Follow-up and longer term care

Health service efficiency necessitates eliminating unnecessary follow-up. However, GWBD or those receiving anti-coagulant therapy are more likely to be resistant to conventional gynaecological management.[32] Prolonged follow-up or patient-initiated follow-up may be necessary to monitor clinical outcomes and blood parameters. Fertility counselling and pregnancy planning may also become necessary for patients of reproductive age.

Patient education and support

Written and verbal education should be provided to patients regarding their diagnosis and treatment. Information on recognising signs of bleeding (or thrombosis) should be given in addition to reinforcing the need to declare their

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diagnosis when presenting with a complication, for example, to healthcare personnel in the ED. Fertility counselling may also become necessary for patients of reproductive age. Planning of pregnancy is imperative in women with bleeding and thrombotic disorders and pre-pregnancy counselling appointments should be offered to give appropriate advice on this, as supported by NICE and the National Health Service (NHS) England Maternal Medicine Network[37,38] Psychological support and counselling should be offered as those living with these conditions can have low self-esteem, feel socially isolated and experience anxiety or depression. Patients should be directed to relevant peer support groups and advocacy services. Stigma and menstrual taboos should be proactively addressed and challenged.

EVALUATING AND ENSURING A HIGH QUALITY MULTIDISCIPLINARY HAEMATOLOGY AND GYNAECOLOGY SERVICE

Patient experience

Women of all ages may feel vulnerable when attending these clinics. Their experience should be evaluated so that continual improvements can be made. This could be done in form of a questionnaire or survey, sent to the patient by text or email following their appointment. Anonymising the respondents would provide the most useful feedback. An example is the NHS Friends and Family test [https://www.england.nhs.uk/fft/].

Governance

Steps should be taken to ensure that a high quality service is provided. This includes regular MDT discussion of cases and patient pathways, incident reporting and root cause analysis and training/education for MDT members.

Audit and quality improvement

Regular audit should take place. This could include review of compliance with referral criteria, evaluation of patient outcomes, feedback from referrers and frequency of non-attendance at clinic reviews.

Research

The development of research studies, preferably in collaboration with national and international networks, would help to address the relative lack of research in GWBD and menstrual issues. These should be guided by areas of need identified by patients and carers as being of importance. Topics might include:

- Development of a registry/database of patients with standardised patient report outcome measures to assess medical treatments, surgical interventions, quality of life (QoL), morbidity, fertility, etc. to provide a longitudinal cohort for comparison with non-IBD/anti-coagulated historical cohorts.
- Design of observational studies and randomised controlled trials to evaluate interventions in an IBD/anti-coagulated population.
- Characterisation of HMB in rare bleeding disorders and Bleeding Disorder of Unknown Cause (BDUC).
- Development of QoL tools to assess the impact of HMB.
- Evaluation of tools to assist with earlier identification of patients with IBDs presenting with HMB to primary care.

Given that the scope of a multidisciplinary haematology and gynaecology service is broad, inclusion and exclusion criteria for research studies should be clearly defined to ensure that these lead to relevant conclusions with which to guide practice.

CONCLUSIONS

A multidisciplinary haematology and gynaecology service aims to place the individual with a bleeding or thrombotic disorder at the centre of care, avoiding repeated hospital visits, duplication of investigations and allowing prompt access to clinical expertise. Timely diagnosis and effective treatment seek to improve patient satisfaction and outcomes including the ability to undertake normal activities of daily living and attend school or work.

This good practice paper promotes the development of a combined service appreciating that it is likely to be of greater value in a tertiary care setting due to the higher volume of complex patients and the centralisation of bleeding disorders services.

***Recommendations*:**

- **We consider that women and girls with bleeding/thrombotic disorders or requiring anti-coagulant therapy may benefit from the development of a multidisciplinary haematology and gynaecology service. (2C)**
- **We would encourage that eligibility criteria for referral to the service are clear, documented and shared with potential referrers. (2D)**
- **We suggest that patients are seen jointly by a haematologist and gynaecologist and are supported by a clinical nurse specialist. (2D)**
- **We suggest that management decisions and peri-procedural planning are communicated clearly to other healthcare providers involved in the care of the**

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individual, with patients receiving copies of all correspondence. (2D)
- **Patient health outcomes and satisfaction should be monitored to ensure the quality of the service that is provided. (2D)**
- **We propose that opportunities for data collection and research are optimised in order to address the relative lack of knowledge in this clinical area. (2D)**

REVIEW PROCESS

Members of the writing group will inform the writing group Chair if any new pertinent evidence becomes available that would alter the strength of the recommendations made in this document or render it obsolete. The document will be archived and removed from the BSH current guidelines website if it becomes obsolete. If new recommendations are made an addendum will be published on the BSH guidelines website (https://b-s-h.org.uk/guidelines/).

DISCLAIMER

While the advice and information in this guidance are believed to be true and accurate at the time of going to press, neither the authors, the BSH nor the publishers accept any legal responsibility for the content of this guidance.

AUTHOR CONTRIBUTIONS

Tina Biss: Conceptualization; investigation; writing – original draft; writing – review and editing; visualization; supervision. **Emily Millen:** Conceptualization; investigation; writing – original draft. **Priyanka Raheja:** Conceptualization; investigation; writing – original draft. **Jayashree Motwani:** Conceptualization; investigation; writing – original draft. **Gillian Lowe:** Conceptualization; investigation; writing – original draft. **Sarah Garside:** Conceptualization; investigation; writing – original draft. **Thomas J. Clark:** Conceptualization; investigation; writing – original draft. **Anna Stone:** Writing – review and editing. **Cora Warren:** Conceptualization; writing – original draft.

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CONFLICT OF INTEREST STATEMENT

All authors have made a declaration of interests to the BSH and Task Force Chairs, which may be viewed on request. The following authors have undertaken advisory board, educational grant and speaker's fees: Tina Biss (Bayer, CSL Behring, Roche-Chugai), Thomas J. Clark (Gedeon Richter, Hologic, Medtronic, Theramex), Gillian Lowe (AbbVie, Amgen, CSL Behring, Sanofi, Sobi), Emily Millen (Roche-Chugai, Sobi), Jayashree Motwani (Bayer, CSL Behring, Roche, Sobi), Priyanka Raheja (BioMarin, CSL Behring, Pfizer, Roche-Chugai, Sobi, Takeda). Sarah Garside, Anna Stone and Cora Warren have no conflicts of interest to declare.

DATA AVAILABILITY STATEMENT

The data that support the findings of this study are available from the corresponding author upon reasonable request.

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SUPPORTING INFORMATION

Additional supporting information can be found online in the Supporting Information section at the end of this article.

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